



FATHER MULLER MEDICAL COLLEGE HOSPITAL LABORATORY

(A unit of Father Muller Charitable Institutions)

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MC-6223

Mr. Sudhakar Gopal Hegde

61 Y MALE

REFERRING PHYSICIAN

DR. GANESH M K

DEPARTMENT

SURGICAL GASTROENTEROLOGY

UHID



250067923

ENCOUNTER NO



IP 902508040242

REQUEST NO

23080019127

Histopathology No

H/8290-8292/25

Report Status

Final

REQUESTED DATE

05/08/2025 09:57:23

COLLECTED DATE

05/08/2025 13:49:36

RECEIVED DATE

05/08/2025 13:49:48

REPORTED DATE

07/08/2025 15:22:45

DEPARTMENT OF HISTOPATHOLOGY

Test Name : BIOPSY PARAFFIN METHOD PER 5 BLOCKS

	Results
Biopsy No :	H/8290-8292/25
Nature of specimen :	8290: Rectosigmoid growth, 8291: Proximal rectal polyp, 8292: Distal rectal growth
Gross :	Grossed by: Dr. Ravindran - Resident doctor; Supervised by: Dr. Reshma: 8290: (Rectosigmoid growth): Received six pale white to dark brown tissue bits largest measuring 0.5cm and smallest measuring 0.2cm in diameter. Tissue all embedded. 8291: (Proximal rectal polyp): Received two pale brown polypoidal tissue measuring 0.6x0.5cm. Tissue all embedded. 8292: (Distal rectal growth): Received six pale white to pale yellow tissue bits largest measuring 0.6cm in diameter and smallest measuring 0.2cm. Tissue all embedded.
Report Status :	Final Report
Microscopy :	8290, A/25, 8291/25, 8292, A/25: 8290, A: (Rectosigmoid growth): Sections studied show colonic mucosa and bits composed of invasive tumor. The tumor cells are arranged in tubulovillous pattern with complex architecture. The tumor cells have penicillate to pleomorphic vesicular nuclei, inconspicuous nucleoli and scant cytoplasm. 8291: (Proximal rectal polyp): Section studied shows a polypoidal neoplasm composed of tubular glands with stratification with low-grade dysplasia. 8292, A: (Distal rectal growth): Sections studied show rectal mucosa and an invasive tumor composed of tumor cells arranged in tubulovillous pattern. These tumor cells have penicillate to pleomorphic vesicular nuclei, inconspicuous nucleoli and scant cytoplasm.
Impression :	8290: (Rectosigmoid growth): Features are of well-differentiated

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adenocarcinoma.

8291: (Proximal rectal polyp): Features are of tubular adenoma with low-grade dysplasia.

8292: (Distal rectal growth): Features are of well-differentiated adenocarcinoma.

Note :

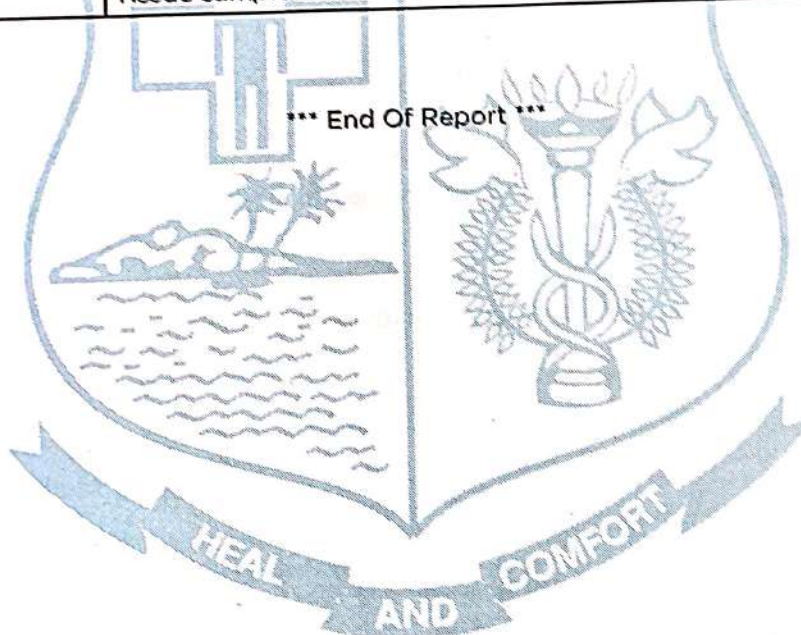
Tissue sample will be discarded after thirty days.

*** End Of Report ***

Verified By

36642

07/08/2025 10:12:46



Slamkar

DR. UMASHANKAR T MBBS, M
CLINICAL PATHOLOGIST

Patient ID	MR661991	Patient Name	Mr.SUDHAKAR GOPAL HEGDE
Gender	Male	Age	63Y
Study Date	04-08-2025 11:37	Study Type	CT
Ref Physician	Dr SWASTHIK UPADHYA P	Order No	

Contrast Enhanced 18F-FDG Whole Body PET-CT Scan

PROTOCOL: 4.7 mCi 18F-FDG was injected after 5-6 hrs fasting. PET-CT scan was performed 45-60 min after the injection on SIEMENS Biograph mCT 20 Time of Flight PET-CT scanner. Semi-quantitative analysis of FDG uptake was performed by calculating SUV value based on body weight. A separate breathhold CT was also acquired. Extent of the scan: Vertex to mid-thigh. The fasting blood sugar level at the time of injection was 157 mg/dl.

INDICATION: Case of biopsy proven carcinoma left mandibular alveolus with sigmoid colon thickening and suspicious hepatic lesion ? dual malignancy under evaluation.

FINDINGS:

BRAIN:

No focal FDG uptake or obvious enhancing lesion noted in the brain parenchyma. Ventricles and CSF spaces appear unremarkable. No mass effect or midline shift noted.

NECK:

The nasopharynx, oropharynx and hypopharynx appear unremarkable. The tonsillar fossa appears unremarkable. The thyroid appears normal.

The IJV and carotid arteries appear unremarkable.

Increased FDG uptake is noted in the erosive lesion with associated soft tissue component involving the body of mandible on the left side, measuring 1.3 x 1.5 cm, SUV max 8.47

Non-FDG avid subcm sized left level IB cervical node is seen, measuring 6 mm.

THORAX:

No lung nodules seen.

No significant mediastinal lymphadenopathy noted.

No pleural effusion is seen.

The heart and mediastinal vessels appear unremarkable.



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No pericardial effusion noted.

ABDOMEN:

Increased FDG uptake is noted in multiple hypodense lesions involving both the lobes of liver, largest measuring 5.4 x 5.1 cm, SUV max 13.95.

● IHBR dilatation noted. The portal vein appears normal.

The stomach appears unremarkable.

Gall bladder, CBD, pancreas, spleen appear normal.

Bilateral adrenals appear normal.

Bilateral kidneys appear normal in size and shape.

Left renal calculus measuring 10 mm is seen.

The descending aorta and IVC appear unremarkable.

Increased FDG uptake is noted in the circumferential wall thickening involving the sigmoid colon (Max Wall Thickness: 2.1 cm, Length: 6.9 cm, SUV max 18.73), causing narrowing of the lumen.

Faint FDG uptake is noted in few subcm sized pericolonic nodes, largest measuring 7 mm, SUV max 1.68

PELVIS:

Mild prostatomegaly is noted.

Urinary bladder appears partially distended.

No ascites is noted.

MUSCULOSKELETAL SYSTEM:

Rest of the axial or appendicular skeleton appears unremarkable.

Rest of the whole body scan appears unremarkable and shows physiological tracer distribution.

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IMPRESSION:

Case of biopsy proven carcinoma left mandibular alveolus with sigmoid colon thickening and suspicious hepatic lesion ? dual malignancy under evaluation.

Scan findings reveal:

- Metabolically active disease is noted in the erosive lesion with associated soft tissue component involving the body of mandible on the left side - *site of biopsy proven primary.*
- Non-FDG avid subcm sized left level IB cervical node is seen - *? metastatic ? reactive.*
- Metabolically active disease is noted in the circumferential wall thickening involving the sigmoid colon, causing narrowing of the lumen - *Possibility of metachronous primary colon malignancy appears most likely. Suggested histopathological confirmation.*
- Faint metabolic activity is noted in few subcm sized pericolic nodes - *likely to be metastatic.*
- Left renal calculus is seen.
- No evidence of metabolically active disease elsewhere in the body.

Sujith Rai

Dr. Sujith Rai

Consultant, PET-CT & Nuclear Medicine

